Fiscal Year:	FY 2018	**Task Last Updated:**	FY 02/19/2018
PI Name:	Roberts, Donna M.D.		
Project Title:	Human Cerebral Vascular Autoregulation and Venous Outflow in Response to Microgravity-Induced Cephalad Fluid Redistribution		
Division Name:	Human Research		
Program/Discipline:	HUMAN RESEARCH		
Program/Discipline--Element/Subdiscipline:	HUMAN RESEARCH--Biomedical countermeasures		
Joint Agency Name:		**TechPort:**	No
Human Research Program Elements:	(1) **HHC**:Human Health Countermeasures		
Human Research Program Risks:	None		
Space Biology Element:	None		
Space Biology Cross-Element Discipline:	None		
Space Biology Special Category:	None		
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Zip Code:	29425-8905	**Congressional District:**	6
Comments:			
Project Type:	Flight	**Solicitation / Funding Source:**	2011 Crew Health NNJ11ZSA002NA
Start Date:	05/16/2013	**End Date:**	09/30/2019
No. of Post Docs:	1	**No. of PhD Degrees:**	0
No. of PhD Candidates:	2	**No. of Master' Degrees:**	0
No. of Master's Candidates:	0	**No. of Bachelor's Degrees:**	0
No. of Bachelor's Candidates:	1	**Monitoring Center:**	NASA JSC
Contact Monitor:	Norsk, Peter	**Contact Phone:**	
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Flight Program:	ISS		
Flight Assignment:	NOTE: End date changed to 9/30/2019 per NSSC information (Ed., 2/26/19)		
Key Personnel Changes/Previous PI:	February 2018 report: added Dr. Bloomberg as Co-Investigator.		
COI Name (Institution):	Brown, Truman Ph.D. (Medical University of South Carolina) George, Mark S M.D. (Medical University of South Carolina) Bloomberg, Jacob Ph.D. (NASA Johnson Space Center)		
Grant/Contract No.:	NNX13AJ92G		
Performance Goal No.:			
Performance Goal Text:			

Task Description:	NOTE this is continuation of "Human Cerebral Vascular Autoregulation and Venous Outflow in Response to Microgravity-Induced Cephalad Fluid Redistribution" grant #NNX13AC32G, which ended 5/15/2013. According to the NASA Human Research Roadmap (Risk of Microgravity-Induced Visual Impairment/Intracranial Pressure; Gaps VIIP2, VIIP4, VIIP5, VIIP8) and further expounded upon in the Visual Impairment Intracranial Pressure (ICP) Summit Report (May 2011), an essential gap exists in our knowledge of intracranial pressure regulation in spaceflight. This is exemplified by the fact that there are few published studies investigating the structural and functional changes occurring as a result of long-term spaceflight and there is almost no data concerning cerebral autoregulation in astronauts following return from space. Only two published studies exist investigating cerebral autoregulation in astronauts using an indirect method for measuring cerebral blood flow (CBF) at the level of the major arteries of the circle of Willis. Thus, although cerebrovascular hemodynamics plays a crucial role in ICP regulation and also in the response to orthostatic stress, we have almost no knowledge of the state of cerebral autoregulatory mechanisms during spaceflight. The operational relevance of this gap according to the NASA Human Research Roadmap is that elevated ICP and likely associated vision disturbances due to microgravity exposure are serious health risks for astronauts. Furthermore, this is an even greater concern for potential longer duration missions such as to the moon or Mars where decreased vision may impact mission performance and may even require early mission termination. Permanent visual losses may result in lifetime disability issues. Almost certainly, during long-term flights, as the hydrostatic pressure gradients due to gravity are abolished with resultant large cephalad fluid shifts, an intracranial adaptive response occurs. Thus direct measurements of cerebral fluid shifts and cerebral hemodynamics as proposed in this study will allow us to more fully characterize the intracranial compartment guiding the development of preventive measures, such as specific pharmacological countermeasures, to combat the development and sequelae of increased ICP during future missions. In order to fully characterize intracranial adaptation to long-term microgravity, we will use magnetic resonance imaging (MRI) which is the terrestrial gold standard. MRI allows direct visualization and functional characterization of the intracranial contents. Indeed, MR imaging is the clinical standard of care in evaluating patients with idiopathic intracranial hypertension. We will use a suite of advanced MRI sequences to: (1.) characterize weightlessness-induced intracranial compartmental fluid volume changes at the macroscopic (brain, CSF) and vascular (arterial and venous) levels following adaptation to long-term spaceflight and (2.) assess for potential resultant hemodynamic changes (cerebral perfusion) in the brain. This proposal represents the most relevant and advanced suite of MRI techniques for the characterization of intracranial compartmental fluid and hemodynamic changes induced by long-term microgravity. The specific sequences will be carried out by the recognized experts in each area and in most cases, the developer of the specific technique. It is imperative to obtain this data in order to have an understanding of what is happening to the brain and orbits while living and working in microgravity. We are only now realizing the consequences of this lack of knowledge. This data will serve as the basis for the development of fundamental theories describing intracranial adaptation to microgravity. It may help establish a risk stratification strategy based on intracranial anatomy, dural venous sinus and jugular vessel outflow, and cerebral vascular autoregulation. Importantly, it will serve as a basis for interpretation of in-flight data collected via other techniques such as ultrasound, the primary in-flight imaging modality.
Rationale for HRP Directed Research:	
Research Impact/Earth Benefits:	Idiopathic intracranial hypertension, or pseudotumor cerebri, is a poorly understood clinical condition of unknown etiology. It has a reported incidence of between 1 and 5 cases per 100,000 people in the general population, although the true incidence may be larger. There is a strong female predominance. Patients typically present with disabling headaches and vision changes. In more extreme cases, injury to the optic nerve can occur and patients can experience vision loss. Upon testing, these patients have increased intracranial pressure and swelling of the optic nerves. Similar symptoms can occur in astronauts following long-term spaceflight. This study will provide a unique opportunity to investigate the underlying pathophysiology of abnormal intracranial pressure regulation which will be applicable to those who suffer from this Earth-based condition. This research may also shed light on the pathophysiology of other Earth-based disease processes associated with abnormal cerebral spinal fluid (CSF) flow or venous outflow obstruction such as obstructive hydrocephalus, normal pressure hydrocephalus and multiple sclerosis, and pathologies associated with altered cerebral autoregulation such as traumatic brain injury.
Task Progress:	We completed a quantitative retrospective review of the pre- and post-flight brain MRI scans of the International Space Station (ISS) and Shuttle astronauts. This included an automated analysis performed using various open source software packages to quantify the structural brain changes associated with long-term spaceflight previously identified qualitatively by radiologist interpretation. The results provide significant insight into the adaptation of the human central nervous system to spaceflight and have important implications for planning a human mission to Mars. We also collaborated with another investigator (Seidler) in the jointly sponsored NASA-German space agency bedrest study performed at the Envihab facility in Cologne, Germany. This collaboration will allow us to obtain additional data concerning the physiological changes of the brain following long-term head down tilt bedrest in a CO2 environment as an analog for spaceflight. The data collection has been completed and we have submitted a request to the Johnson Space Center (JSC) Institutional Review Board (IRB) and our local Medical University of South Carolina (MUSC) IRB for approval to analyze the data. In addition, our project team has participated in the annual NASA Human Research Program Investigators Workshops held in Galveston (January, 2018). As an outreach effort, Dr. Roberts participated in the South Carolina Palmetto Academy Summer Program, funded through the South Carolina Space Grant Consortium, by mentoring a summer undergraduate student and a medical student who assisted with analysis of preliminary data for this study.
Bibliography Type:	Description: (Last Updated: 01/15/2026)
Articles in Peer-reviewed Journals	Roberts DR, Albrecht MH, Collins H, Asemani D, Chatterjee AR, Spampinato MV, Zhu X, Chimowitz MI, Antonucci MU. "Effects of spaceflight on astronaut brain structure as indicated on MRI." N Engl J Med. 2017 Nov 2;377(18):1746-53. https://doi.org/10.1056/NEJMoa1705129 ; PubMed PMID: 29091569 , Nov-2017
Articles in Peer-reviewed Journals	Roberts DR, Antonucci MU. "Effects of spaceflight on astronaut brain structure. [Author reply]" N Engl J Med. 2018 Feb 8;378(6):582-3. https://doi.org/10.1056/NEJMc1716067 ; PubMed PMID: 29414278 , Feb-2018

Books/Book Chapters	Roberts DR, McLaren T, Antonucci MU. "Advanced Imaging of the Intracranial Physiology of Spaceflight." in "Vision Impairment: Fluid Shifts in Microgravity, Intracranial Pressure and Its Effect on Vision in Space and on Earth." Ed. B.R. Macias, A.R. Hargens, J.H.K. Liu, C. Otto. Hackensack, N.J. : World Scientific, 2017. p. 173-224. ISBN-13: 978-9814667104 https://doi.org/10.1142/9789814667111_0009 , Aug-2017
Significant Media Coverage	Roberts DR, Albrecht MH, Collins H, Asemani D, Chatterjee AR, Spampinato MV, Zhu X, Chimowitz MI, Antonucci MU. "Research summary of article, 'The Effect of Spaceflight on Human Brain Structure by MRI,' N Engl J Med. 2017 Nov 2;377(18):1746-1753, highlighted in the news media world-wide." News media world-wide. For example, https://www.cnn.com/2017/11/01/health/brain-space-astronaut-mri-scan-study/index.html , Nov-2017